



SCGOPHCG Clinical Practice Guideline

These guidelines are intended to guide clinical decision making for clinical staff at North Metropolitan Health Service. They are not strict protocols and do not replace clinical judgement. These guidelines should never be relied on as a substitute for proper assessment of the particular circumstances of each case and the needs of each patient.

If staff decide, in exercising their clinical judgment, not to follow the guideline, it is recommended that the reasons are clearly documented in the patient's health care record.

Gastrointestinal Endoscopic Procedures

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

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Introduction

This guideline outlines the procedures and responsibilities of both individuals and the health service organisation in relation to gastrointestinal endoscopic procedures performed at Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG). It ensures the delivery of a uniform, integrated and multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline may:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☒
Other:	☐		

1.0 General Information

Gastrointestinal endoscopy procedures involve the direct visual examination of the lumen of the gastrointestinal tract. Gastrointestinal endoscopy procedures are demonstrated to be an efficient and effective way of evaluating both the appearance and integrity of patient's gastrointestinal mucosa. In doing so, it allows for the purpose of diagnosis and provides access for therapeutic procedures

Refer to [Appendix 1: Table of Endoscopic Procedures](#) to see specific procedures performed at SCGOPHCG.

1.1 Referral and Initial Assessment

A 'referral' is required for all procedures to be performed at SCGOPHCG. Prior to the patient's scheduled procedure, all referrals undergo a clinical review to determine the patient's suitability for care, urgency categorisation, and specific procedural requirements (Example; if a patient needs specific equipment, or additional anaesthetic support). The patients are categorised as per the Department of Health Urgency Categorisation and Access Policy.

The triaging clinician should take into consideration Osborne Park Hospital (OPH) medical exclusion criteria and equipment availability when assigning site for scope. Refer to Speciality Area Admission Guideline (SCGOPHG).

Currently, as of July 2024, gastroenterology procedure e-referrals are not accepted at OPH. Medical Staff requesting a gastrointestinal endoscopy procedure for current OPH inpatients, should contact:

- the OPH Endoscopy Waitlist Co-Ordinator on 6457 8189, or alternatively
- the OPH endoscopy Clinical Nurse Manager on 6457 8102.

The Endoscopy Waitlist Coordinator will advise which list is available, and the Gastroenterologist Consultant to contact.

Sir Charles Gairdner Hospital (SCGH) utilises e-Referrals for 'Urgent' in-patients following consultation with a Gastroenterology Registrar or Gastroenterology Consultant.

Please note: It is the responsibility of the treating team RMO/Registrar that is requesting the inpatient procedure to:

- obtain fasting and bowel preparation instructions.
- communicate with the ward shift coordinator and relevant staff to ensure all are informed of procedural requirements, including the scheduled procedural date, preparation and document in the medical record.

- obtain patient consent for the procedure as per SCGOPHCG 'Consent to Treatment' policy.

2.0 Colonoscopy

2.1 Patient Safety Information

Clinical Alerts:

- Clinical staff are to ensure correct and complete reporting, notification and documentation of clinical/medical alerts. RED patient identification bands are to be used for patients with documented allergies. Refer to Clinical Alerts Policy (SCGOPHCG).

Diabetic Patients:

- Gliflozins should be withheld at least 3 days pre-procedure (2 days prior to procedure and the day of procedure) Refer to SGLT2 Inhibitors (Gliflozins) and Euglycaemic Ketoacidosis Guideline (SCGOPHCG).
- GLP-1 Agonists are not to be withheld prior to procedure⁷
- OPH only Patient Alert form is completed; patient alert sticker affixed in the left column, detailed description such as: Diabetes Mellitus Type 1, list of medication and/or insulin.

Management of Anticoagulants:

- Considerations should be made by the clinical team when triaging referrals- determining the time of anticoagulation interruption and whether bridging coagulation is required. Recommendations based on assessment of the patient should be flagged with endoscopy nurse shift coordinator. Refer to Periprocedural Management of the Anticoagulants Guideline (SCGOPHCG).
- Heparin infusions must be ceased 6 hours prior to procedure.
- Subcutaneous anticoagulants must be withheld on the morning of procedure.

HCG/Pregnancy testing: (OPH Only)

- All female patients between the ages of 16 and 50 years, and any women over 50 years who continue to menstruate require HCG/pregnancy testing pre-procedure.

2.2 Medical Requirements

Pre-Procedure:

- Ensure patient has a patent peripheral intravenous catheter (PIVC), documentation of PIVC insertion date, time, size and placement. Refer to Peripheral Intravenous Catheters (PIVC) Insertion, Maintenance and Removal Clinical Practice Guideline (SCGOPHCG).
- Consent to Treatment completed in accordance with WA Health Consent to Treatment Policy
- Anaesthetist review if appropriate. Blood tests– full blood count, electrolytes, coagulation profile where appropriate
- Electrocardiograph (ECG) 12 lead – if patient has significant cardiac history
- Medical Officer is to review medication to be withheld.

Post-Procedure:

- Ensure any post procedure orders or instructions have been documented on the procedure report.
- If required, ensure that a written medical prescription has been documented for oxygen therapy. Refer to Oxygen Therapy Clinical Practice Guideline (SCGOPHCG).

2.3 Nursing Management**Pre-Admission: Outpatient Education**

- The patient will be contacted via telephone by endoscopy pre-admission nurse prior to scheduled appointment.
- The pre-admission nurse will provide the patient an opportunity to discuss the procedure, anaesthesia, bowel preparation, medication management etc
- The patients understanding of the procedure will be assessed, available resources and services can be accessed to assist the patient including interpreter and Aboriginal Liaison Services.

Pre-Procedure (On admission)

- Nurse is to administer any medications required, as per Medical Orders.
- All patients require nurse escort to and from the ward/department/procedural area. Clinical handover using iSoBAR must be provided pre and post procedure. Refer to Clinical Handover Policy (SCGOPHCG).
- Patients core physiological observations signs are to be recorded on the Observations and Response Chart. Refer to Recognising and Responding to Acute Deterioration Procedure (SCGOPHCG).
- Document the results of any anti-coagulation therapy monitoring.
- Patients with diabetes are to have their blood glucose level taken on admission to the department and documented on the appropriate chart to minimise risk of hypoglycaemic episode. Blood Ketone Testing is to be initiated for all patients on SGLT2 medications. Refer to Diabetes Patient Management Nursing Practice Guideline (SCGOPHCG).
- All relevant clinical documentation must accompany the patient including:
 - Medical records
 - Consent to Treatment form
 - Procedural Checklist Form
 - Observation and Response chart
 - Patient Safety Checklist (OPH)
 - Anaesthetic record (if applicable)
 - Anaesthetic Consent Form (if applicable)
- Ensure the patient has fasted pre-procedure.

- See Appendix 1 for relevant fasting and Sip till Send instructions.
 - Patients prescribed Glucagon-like peptide-1 receptor agonists (GLP-1RAs) such as Semaglutide (Ozempic, Wegovy), Liraglutide (Victoza, Saxenda), Dulaglutide (Trulicity) and Tirzepatide (Mounjaro) have different fasting recommendations due to delayed gastric emptying.⁷
 - See Appendix 2 for relevant fasting times.
- AM Procedure session commences 0800hrs.
- PM Procedure session commences 1300hrs.

- Ensure the patient has completed adequate bowel preparation. It is essential that the bowel fluid is pale yellow in colour with no solid particles prior to transfer to department/procedural area.
- If there are any concerns regarding patient preparedness contact:
 - OPH - Procedure Room 1 ext. 78094, Procedure Room 2 ext. 78166.
 - SCGH – Shift Co-ordinator ext. 74523

- Patient to wear theatre gown

- Remove or tape jewellery

OPH: The patient will be transported on a patient trolley or DPU bed

SCGH: The patient will be transported on their bed or a G75 trolley.

Intra-Procedure

- Follow standard and transmission-based precautions with appropriate use of personal protective equipment.
- Team is to ensure procedure matching policy is followed including correct patient, correct site, correct procedure. 'Team Time Out' is to be documented on the SCGOPHCG Procedural Checklist
- Continuous observation of the patient is required to ensure patients safety and to assess appropriate level of sedation.
 - Core observations including respiratory rate, oxygen saturation, heart rate, blood pressure and level of consciousness are to be recorded at a minimum of 10 minutely intervals using the Observation and Response Chart.
 - Additional monitoring including ECG, capnography should be available if required.
- There should be a minimum of three clinicians available during procedural sedation, the proceduralist who is a registered medical officer, and two suitable qualified assistants one of whom is designated as sedation assistant. Any additional staff requirements will be determined by the intended level of sedation and complexity of the procedure⁶

Post Procedure

- Procedural Intravenous Sedation (OPH): Core observations including level of consciousness are to be recorded on arrival to Day Procedure Unit, if within accepted parameters record every 30 minutes for 1 hour, then 1-hourly for 2-hours.
- Procedural Intravenous Sedation (SCGH): Core observations including level of consciousness are to be recorded on arrival to Recovery/ PACU, if within accepted parameters record every 15 minutes for 1 hour, then 1-hourly for 2-hours.
- Patient observation requirements may change as indicated by type of anaesthetic administered or patient complexity.
- Assess the patient for signs of complications such as:
 - Bleeding
 - Pain
- Nutrition and Hydration: Colonoscopy patients can have diet and fluids once they are conscious. Resume diet as tolerated unless doctor instructs otherwise.
- **Inpatients:** following the procedure the medical team and nursing are to assess the need for the PIVC with the view to remove the PIVC when clinically indicated or no longer required.
- **Inpatients:** Monitor the insertion site and perform a peripheral intravenous assessment score (PIVAS) for the duration of the PIVC in situ and for 48 hours post removal.

3.0 Colonic Lavage

3.1 Legislative Requirements

- All prescriptions are to meet the minimum requirements set under the Medicines and Poisons Act 2014 and Medicines and Poisons Regulations 2016, and SCGOPHCG Prescribing Policy and Procedure in addition to best practice requirements.^{7,8,9,10.}
- Only health professionals authorised under the Health Practitioner Regulation National Law, and acting in the lawful practice of their profession, can prescribe medicines. Only valid prescriptions will be utilised by any nursing staff to administer medication except for verbal orders and nurse initiated medications, where a valid and approved Structured Administration and Supply Arrangement (SASA) is in place.

3.2 Safety Information

The goal of bowel preparation for colonoscopy, is to clean the colon for effective examination in a safe and tolerable manner.

No more than 5 litres of colonic lavage should be given without medical review.

3.3 Medical Requirements

- Notify Medical Officer if patient is bleeding per rectum.
- A valid prescription is required prior to nursing staff administering medication.
- OPH only: Patients being admitted to OPH surgical ward for bowel preparation will require colonic lavage prescribed by Gastroenterology consultant prior to admission. Surgical RMO will complete the patient admission and can be contacted for any clinical escalation or clinical concerns.
- Specific written instructions regarding bowel preparation are required for any patient with:
 - Significant cardiac history,
 - Renal or pulmonary disease,
 - Diabetes,
 - Bowel obstruction or colitis,
 - Cystic Fibrosis,
 - Previous unsuccessful bowel preparation.
- Three (3) Days before the examination **cease** taking constipating agents such as:
 - Oral iron tablets
 - Panadeine (or forte)
 - Codeine phosphate

3.4 Nursing Management: Outpatients

- Unless directed by the triaging clinician, patients will be sent via post/email bowel preparation instructions for PLENVU. For more information refer to Appendix 3: PLENVU Timeline

Extended bowel preparation can be recommended if the 'normal' (Plenvu) bowel preparation did not yield adequate results in the past, or if the patient experiences constipation. Clinician will provide patient with Colonlytely bowel preparation.

3.5 Nursing Management: Inpatients

Day Prior to Procedure

- On the morning of the day before the examination, commence clear fluid diet. Refer to Preoperative Care Clinical Practice Guideline (SCGOPHCG) for the list of approved Clear Fluids.
- Administer Bisacodyl tablets as prescribed.
- Mix 4 Litres of colonic lavage (Colonlytely) solution.
- AM cases – Commence drinking 3 litres at 16:00PM
- PM cases – Commence drinking 2 litres at 18:00PM
- **Colonic lavage is to be consumed at approximately 1 litre/hour to ensure effective cleansing of colon.**
- SCGH – Patients who cannot tolerate colonic lavage preparation orally should be reviewed by medical staff, and if clinically appropriate, may have it administered via nasogastric tube using continuous infusion at a rate of approximately 1 litre per hour.
- Monitor the patient for any clinical adverse effects, and initiate observations and respond according to escalation protocol.

Day of Procedure

- AM cases: drink 1 litre of colonic lavage solution at 0400hrs then fast from 0600hrs (see Appendix 1)
- PM cases: drink 2 litres of colonic lavage solution at 0800hrs then fast from 1100hrs (see Appendix 1).
- If necessary, another 1 litre of colonic lavage solution is given to achieve a clear output.
- If return not clear, contact:
 - OPH Endoscopy Unit ext. 78094 (room1), or 78166 (room 2).
 - SCGH Endoscopy Suite ext.74523
- Some patients admitted from the community for a booked procedure may advise nursing staff that they have followed preparation instructions which include the consumption of a white diet. A white diet comprises of white or cream-coloured foods of low residue. This diet will have been commenced in the community.
- Inadequate bowel preparation will result in poor visualisation of the mucosa, leading to repeat Colonoscopy or cancelation. Liquid that is watery or yellow and clear indicates that the patients bowel is ready for Colonoscopy.



4.0 Discharge Requirements

For endoscopy outpatients, discharge is based on the following criteria:

- Observations within normal limits
- Awake and Alert

- Tolerating fluids (if not fasting post procedure)
- No significant pain
- Minimal nausea/vomiting/dizziness

This is documented on the SCGOPHCG Procedural Checklist:

- Patients must stay a minimum of 60 mins from the time the last dose of systemic medication or 30 mins following oral medication.
- PIVC must be removed prior to discharge.
- Patients are to be given specific endoscopy discharge information as well as a copy of their procedure report.
- Patients having had sedation or anaesthetic are to be discharged into the care of a responsible adult who must stay with the patient overnight.

Related Documents

- Health Services Act 2016
- Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016
- SCGOPHCG Medicines Management Policy and Procedures
- SCGOPHCG Prescribing Policy and Procedure
- SCGOPHCG Structured Administration and Supply Arrangements (SASA) Policy
- SCGOPHCG Clinical Alert Policy
- WA Department of Health -Urgency Categorisation and Access Policy
- WA Health Consent to Treatment Policy
- SCGOPHCG Infection Prevention and Management Manual, Aseptic Technique Policy.
- SCGOPHCG Infection Prevention and Management Manual, Standard and Transmission Based Precautions Policy
- SCGOPHCG Falls Prevention and Management Clinical Practice Guideline
- SCGOPHCG Removal Clinical Practice Guideline- Peripheral Intravenous Catheters (PIVC) Insertion, Maintenance
- SCGOPHCG Periprocedural Management of the Anticoagulants Guideline.
- SCGOPHCG Guideline - Specialty Area Admission Criteria guideline.
- SCGOPHCG Guideline- Sodium Glucose Co-Transporter2 (SGLT2) - Inhibitors (Gliflozins) And Euglycaemic Ketoacidosis
- SCGOPHCG Clinical Practice Guideline -Oxygen Therapy
- SCGOPHCG Nursing Practice Guideline Pre-Operative Care

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Appendix 1: Table of Endoscopic Procedures performed at SCGOPHCG

Note: For patients on GLP-1RAs undergoing Endoscopy Procedures that require fasting but no bowel preparation (marked with a *) see appendix 2 for alternative fasting instructions.

Refer to Preoperative Care Clinical Practice Guideline (SCGOPHCG) for the list of approved sip til send protocol Clear Fluids.

PROCEDURE	WRITTEN CONSENT	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	LENGTH OF PROCEDURE
Colonoscopy	✓	As per Section 3 – Colonic Lavage Regime and Sip til send	Hospital Gown PIVC Colonic Lavage (see Section 3)	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Observe for pelvic pain Diet as normal	Advise SCGH G75 / OPH procedure room when bowel output is clear	30mins
*Flexible Sigmoidoscopy	✓	6hrs solids Clear Fluids – sip til send	Hospital Gown PIVC 2 X phosphate enemas (1/2 hr apart) Minimum of 1 hour prior to procedure	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Observe for pelvic pain Diet as normal	Advise G75 / OPH procedure room when enemas administered	15mins
*Gastroscopy	✓	6hrs solids Clear Fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Nil orally for 1 hr or until gag reflex back		15mins
*Gastroscopy + Dilatation	✓	6hrs solids Clear Fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Nil orally for 4hrs then soft diet overnight unless otherwise indicated on Procedure report		30mins
*Gastroscopy + Variceal Banding (SCGH ONLY)	✓	6hrs solids Clear Fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Nil orally for 4 hrs then clear fluids overnight		30mins

Table of Endoscopic Procedures performed at SCGOPHCG (cont'd)

PROCEDURE	WRITTEN CONSENT	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	LENGTH OF PROCEDURE
*Gastroscopy + Argon Plasma Coagulation (APC) (SCGH ONLY)	✓	6hrs solids Clear Fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Nil orally for 2 hrs then clear fluids overnight		30 mins
*Gastroscopy (+/- Stent, POEM, ESD, EMR) (SCGH ONLY)	✓	6hrs solids Clear fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core Physiological observations	30/60 for 2 hours, then 4/24 To remain Nil orally. See Procedure report for fasting/diet instructions		45mins
*Endoscopic Ultrasound (SCGH ONLY)	✓	6hrs solids Clear fluids - Sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, hourly for 2 hrs, then 4/24 See post procedure orders prior to commencing diet and fluids		30 + mins
*Gastroscopy +PEG insertion (NOT OPH)	✓	6hrs solids Clear fluids – sip til send	Hospital Gown PIVC	Checklist	Baseline core physiological observations	30/60 for 2 hours, hourly for 2 hrs, then 4/24 See NPG # 31 Gastrostomy Tubes for management and care required post procedure.	Patient to shower / bathe using Chlorhexidine 4% wash on the morning of the procedure	30mins
Flexible Cystoscopy	✓	Nil	Hospital Gown Empty bladder	Checklist	Baseline core physiological observations		Transport in wheelchair to G75	15mins

Table of Endoscopic Procedures performed at SCGOPHCG (cont'd)

PROCEDURE	WRITTEN CONSENT	FASTING	PREPARATION	DOCUMENTATION	OBS PRE	OBS POST	NOTE	LENGTH OF PROCEDURE
*E.R.C.P. (SCGH ONLY)	✓	6hrs solids Clear fluids – sip til send	Hospital Gown PIVC in right hand FBC, U&E, Coagulation profile Check last time patient voided	Checklist	Baseline core physiological observations	30/60 for 2 hours, hourly for 2 hrs, then 4/24 Nil orally for 4 hrs, observe for abdominal pain (acute pancreatitis is a known complication). Check with Medical Officer if blood lipase levels are required prior to recommencing oral fluids. Clear fluids only overnight then light diet. See procedure report for more detailed post procedure instructions	Performed in Rm 20 Radiology Specials 1 st floor	45+ mins
*Bronchoscopy (SCGH ONLY)	✓	4hrs solids 4hrs fluids	Hospital Gown PIVC, INR if indicated Administer pre-medication as prescribed	Checklist	Baseline core physiological observations	30/60 for 2 hours, then 4/24 Oxygen to continue as per medical orders Fast as per post procedure orders (usually 2hrs) Assess swallow reflex is present prior to commencing fluids	During this aerosol generating procedure a fit checked N95 or P2 respiratory mask must be worn by all staff	30 mins
Capsule Endoscopy (SCGH ONLY)	✓	10hrs solids 10hrs fluids	2 litres gastric lavage evening prior to examination	Checklist	Baseline core physiological observations	Check stools for 3 days to ensure Capsule excretion. To report capsule has passed, contact G75 nursing staff.	Cease iron supplements 7 days prior. No medication for 2hrs prior. Refer to peri-procedure instruction sheet	30mins



Appendix 2: Alternative fasting requirements for patients on GLP-1RAs prior to Endoscopic Procedures performed at SCGOPHCG⁸

Patients prescribed Glucagon-like peptide-1 receptor agonists (GLP-1Ras) such as Semaglutide (Ozempic, Wegovy), Liraglutide (Victoza, Saxenda), Dulaglutide (Trulicity) and Tirzepatide (Mounjaro) have different fasting recommendations due to delayed gastric emptying.

Before Endoscopy Procedures that require fasting but no bowel preparation, patients on GLP-1RAs are to be directed to follow the recommended fasting status:

- Ensure it is clearly documented on the gastro procedure checklist that patient is prescribed GLP-1RAs
- Enter Clear Fluids as diet type into the Automated Menu System (AMS)

AM session	Clear fluids for the full day before surgery 24:00 – 24:00	Stop clear fluids but continue to drink water the night before at 24:00 until 06:00 (125mls or half a cup each hour)	From 06:00 stop drinking water and start fasting NBM
PM session	Clear fluids for the full day before surgery 06:00 – 06:00	Stop clear fluids but continue to drink water from 06:00 until 11:00 (125mls or half a cup each hour)	From 11:00 stop drinking water and start fasting NBM
Emergency procedure if you are unable to have the 24 hours clear fluids lead in time		Continue to drink water (125mls or half a cup each hour)	From 2 hours before scheduled procedure stop drinking water and start fasting NBM